

OREGON STATE HOSPITAL

POLICY ATTACHMENT

PROCEDURES A: Discharge Readiness

POLICY: 6.013

POINT PERSON: Director of Social Work

APPROVED: Sara Walker, MD
Chief Medical Officer/Interim
Superintendent

**DATE: DECEMBER 24,
2024**

SELECT ONE:

☐ New policy attachment

☒ Minor/technical revision of existing policy attachment

☐ Reaffirmation of existing policy attachment

☐ Major revision of existing policy attachment

RESPONSIBLE PERSON/GROUP	PROCEDURES
Interdisciplinary Treatment Team (IDT)	1. Encourage patient involvement whenever possible in decisions regarding community placement when planning discharge or conditional release. 2. Identify barriers that could impede or delay discharge, and: a. develop Treatment Care Plan (TCP) goals and interventions specific to the identified discharge barrier with identified completion dates, and b. assign IDT resources to mitigate identified barriers. 3. The IDT, Risk Review Panel, or Program Executive Team (PET) (as applicable) determines the patient is “ready to transition” or “conditional release ready.” 4. Document discharge readiness status in the patient’s medical record at the time the IDT determines the patient’s discharge readiness per department protocols. 5. After determining that a patient under the PSRB can be safely supervised in the community, request the Risk Review Panel review and approve or deny a conditional release readiness request. 6. Provide the patient’s guardian with opportunity to participate in discharge planning. 7. If a patient refuses to participate in all or part of the discharge planning process, document the refusal, the reasons for

	refusal, the nature of the plans offered, and efforts made on behalf of the patient. 8. Use clinical judgment and include the patient to identify whom the patient wants to be notified of their discharge. a. Identify who is legally required to be notified of the patient's discharge. b. Notify the identified persons. 9. Assess the patient's risk for suicide in accordance with OSH Policy and Procedure 6.056, "Suicide Risk Screening and Assessment". 10. Include input from the patient's lay caregiver, when applicable, and other supports. 11. If a patient must be unexpectedly discharged, (e.g., due to court action or a voluntary admit patient request), attempt to connect with community-based services and supports for continuing care. Document such efforts.
Psychiatrist/PMHNP	1. For patients under Oregon Revised Statute (ORS) 161.370, complete a Clinical Hospital Level of Care Assessment in collaboration with other members of the IDT at the 10 Day Review and at each subsequent Treatment Care Plan Meeting. For Patients under ORS 426.130 and 426.220 complete a Ready to Transition Assessment in collaboration with Social Work and other members of the IDT at every Treatment Care Plan meeting. 2. For all patients, identify the individual's discharge readiness in a progress note, including barriers to discharge and associated interventions for those not deemed ready. 3. Collaborate with the social worker and patient on discharge planning. 4. Complete the medical record and discharge order, including destination of patient's discharge, and sign the discharge order. 5. Complete the Psychiatry Discharge Summary within 10 days of patient discharge and include the course of hospitalization, continuing care plan, and recommendations for needed treatment.

Registered Nurse (RN)	1. Per Nursing protocol 2,027 "Discharge of Patients," complete the Nursing Discharge Bundle, which includes the Discharge Nursing Assessment and Suicide Risk Screening (Discharge), within 24 hours prior to the patient's discharge. 2. Review the Patient Discharge Instructions report with the patient and caregiver, when possible. 3. Facilitate treatment and medical equipment to send with the patient per Nursing protocol.
Social worker	1. Document community integration and discharge planning progress throughout the patient's hospitalization in progress notes in accordance with applicable protocols. 2. Complete a continuing care discharge plan (CCDP), including information such as housing, supervision, addresses, support systems, persons to contact, medical needs, appointments, special care needs, an evaluation of the likelihood the patient needs post-hospital services and the availability of the services, and recommendations in accordance with ORS 441.196. 3. Schedule follow-up appointments for no later than seven days after discharge 4. If follow-up appointments cannot be scheduled within seven days after discharge, document why the seven-day goal could not be met. 5. Include recommendations for mental health care in the CCDP for a patient going to another institutional setting (e.g., jail, correctional facility, medical hospital) both for that institutional setting and for discharge planning to the community from that setting. 6. If the patient or legal guardian refuses to sign the CCDP, write, "patient (or guardian) refuses to sign" in the open space provided for the signature, and initial the statement. 7. Attach lists of resources specific for the patient's needs which could include skilled nursing facilities (SNF), intermediate care facilities (ICF), local housing, shelters, food banks, etc. 8. For patients under civil commitment or conditionally releasing under the PSRB, ensure transport is arranged. 9. Complete notification to external partners of discharge, including phone contact with the receiving facility or provider, if there are immediate risks identified by the IDT. Document contact.

	10. When completed, sign the CCDP, and send copies of the CCDP form to the patient, care provider, community mental health plan representative, and other involved agencies at discharge.
Health Information	1. When the Psychiatry Discharge Summary is completed, check the CCDP and send out with other identified clinical documents identified by the Psychiatrist/PMHNP to continuing care providers. 2. Document the date of the disclosure(s).